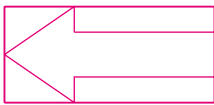
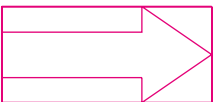
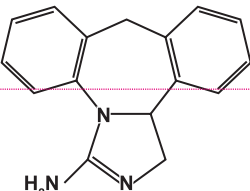


Safety and efficacy in patients ≥ 12 years has been established in clinical trials. RELESTAT™ may be used in adolescents (12 years of age and older) at the same dosage as in adults. The safety and efficacy of RELESTAT™ in children less than 3 years of age have not been established. There are limited data on the safety in children aged 3-12 years (see section Pediatric Use). Hepatic impairment RELESTAT™ ophthalmic solution has not been studied in patients with hepatic impairment. Post-marketing safety data from the tablet formulation of epinastine hydrochloride (up to 20 mg once daily) indicates that the incidence of adverse reactions was higher in this group compared with adult patients without hepatic impairment. The daily dose of a 10 mg epinastine hydrochloride tablet is more than 100-fold higher than the daily dose following RELESTAT™. In addition, the metabolism of epinastine in humans is minimal (<10%). Therefore, no dosage adjustment is considered to be necessary. Renal impairment RELESTAT™ ophthalmic solution has not been studied in patients with renal impairment. Post-marketing safety data from the tablet formulation of epinastine hydrochloride (up to 20 mg once daily) indicate that there are no particular safety issues for patients with renal impairment. As such, no dosage adjustment is considered to be necessary. INTERACTIONS WITH OTHER MEDICAMENTS No specific drug interaction studies have been performed with RELESTAT™ ophthalmic solution. No drug-drug interactions are anticipated in humans since systemic concentrations of epinastine are extremely low following ocular dosing. In addition, epinastine is mainly excreted as unchanged drug in humans which is indicative of minimal metabolism. OVERDOSE AND TREATMENT After instillation of 0.3% epinastine hydrochloride eye drops 3 times daily (which corresponds to 9 times the recommended daily dose), reversible miosis, without influence on visual acuity or other ocular parameters, was observed. The 5 mL bottle of RELESTAT™ ophthalmic solution contains 2.5 mg of epinastine hydrochloride (10 or 20 mg once daily is an oral dose). Intoxication after oral ingestion of the ophthalmic formulation is not expected even if the entire contents of the bottle are swallowed. No case of topical overdose has been reported. HOW SUPPLIED RELESTAT™ ophthalmic solution is supplied sterile in opaque white low density polyethylene (LDPE) plastic bottles with dropper tips and white high impact polystyrene (HIPS) caps as follows: 5 mL fill in 10 mL bottle. Storage: Store below 30°C. Keep the bottle in the outer carton in order to protect from light. Keep out of the reach of children. Shelf Life: 24 months. Discard unused contents four weeks after opening. Date of Revision: August 2023	    RELESTAT™ (epinastine hydrochloride ophthalmic solution) 0.5 mg/mL Sterile
Rx Only Controlled Medicine/Ubat Terkawal Manufactured by Allergan Pharmaceuticals Ireland Castlebar Road, Westport County Mayo Ireland PRODUCT REGISTRATION HOLDER ABBVIE SDN BHD 9th Floor Menara Lien Hoe No.8, Persiaran Tropicana Tropicana Golf & Country Resort 47410 Petaling Jaya Selangor, Malaysia © 2023 AbbVie. All rights reserved. RELESTAT and its design are trademarks of Boehringer Ingelheim International GmbH. 20081064	RELESTAT™ (epinastine HCl ophthalmic solution) 0.05% is a clear, colorless, sterile isotonic solution containing epinastine HCl, an antihistamine and an inhibitor of histamine release from the mast cell for topical administration to the eyes. Epinastine HCl is represented by the following structural formula:  x HCl $C_{16}H_{15}N_3$ HCl Mol. Wt. 285.78 Chemical Name: 3-Amino-9, 13b-dihydro-1H-dibenz[c,f] imidazo[1,5-a] azepine hydrochloride Each mL contains: Active: Epinastine HCl 0.05% (0.5 mg/mL) equivalent to epinastine 0.044% (0.44 mg/mL); Preservative: Benzalkonium chloride 0.01%; Inactives: Edetate disodium; purified water; sodium chloride; sodium phosphate, monobasic; sodium hydroxide and 1N sodium hydroxide and/or 1N hydrochloric acid (to adjust the pH). RELESTAT™ has a pH of approximately 7 and an osmolality range of 250 to 310 mOsm/kg. CLINICAL PHARMACOLOGY Epinastine is a topically active, direct H_1-receptor antagonist and an inhibitor of the release of histamine from the mast cell. Epinastine is selective for the histamine H_1-receptor and has affinity for the histamine H_2-receptor. Epinastine also possesses affinity for the $\delta 1$-, $\delta 2$-, and 5- HT_2-receptors. Epinastine does not penetrate the blood/brain barrier and, therefore, is not expected to induce side effects of the central nervous system. Fourteen subjects, with allergic conjunctivitis, received one drop of RELESTAT™ ophthalmic solution in each eye twice daily for seven days. On day seven average maximum epinastine plasma concentrations of 0.04 ± 0.014 ng/mL were reached after about two hours indicating low systemic exposure. While these concentrations represented an increase over those seen following a single dose, the day 1 and day 7 Area Under the Curve (AUC) values were unchanged indicating that there is no increase in systemic absorption with multiple dosing. Epinastine is 64% bound to plasma proteins. The total systemic clearance is approximately 56 L/hr and the terminal plasma elimination half-life is about 12 hours. Epinastine is mainly excreted unchanged. About 55% of an intravenous dose is recovered unchanged in the urine with about 30% in feces. Less than 10% is metabolized. The renal elimination is mainly via active tubular secretion. Clinical studies: Epinastine HCl 0.05% has been shown to be a significantly superior to vehicle for improving ocular itching in patients with allergic conjunctivitis in clinical studies using two different models: (1) conjunctival antigen challenge (CAC) where patients were dosed and then received antigen instilled into the inferior conjunctival fornix; and (2) environmental field studies where patients were dosed and evaluated during allergy season in their natural habitat. Results demonstrated a rapid onset of action for epinastine HCl 0.05% within 3 to 5 minutes after conjunctival antigen challenge. Duration of effect was shown to be 8 hours, making a twice daily regimen suitable. This dosing regimen was shown to be safe and effective for up to 8 weeks, without evidence of tachyphylaxis.

		Nursing Mothers: A study in lactating rats revealed excretion of epinastine in the breast milk. It is not known whether this drug is excreted in human milk. Because many drugs are excreted in human milk, caution should be exercised when RELESTAT™ ophthalmic solution is administered to a nursing woman. Pediatric Use: Frequency, type and severity of adverse reaction in adolescents ≥ 12 years of age are expected to be the same as in adults. There is limited experience in children 3-12 years of age regarding frequency, type and severity of adverse reactions. A 6-week, randomised, double-masked, vehicle controlled study (2:1) involving 96 ocular-wise non-symptomatic, healthy children aged 3-12 years, indicated that RELESTAT™ was well tolerated and did not identify any significant differences between the groups for any safety variable. Treatment related reactions were conjunctival follicles (6.3% in both epinastine and vehicle- treated subjects) and conjunctival hyperaemia (1.6% of epinastine treated subjects and none in the vehicle group). Safety and efficacy in patients ≥ 12 years has been established in clinical trials. Geriatric Use: RELESTAT™ ophthalmic solution has not been studied in patients older than 65 years of age. Postmarketing safety data from the tablet formulation of epinastine hydrochloride (up to 20 mg once daily) indicates that there are no particular safety issues for elderly patients compared with younger adult patients; therefore, no dosage adjustment is considered necessary. EFFECTS ON ABILITY TO DRIVE AND USE MACHINES As with any ocular medication, if transient blurred vision occurs at instillation, the patient should wait until the vision clears before driving or using machinery. ADVERSE REACTIONS The most frequently reported adverse reaction occurring in approximately 5% of patients was burning sensation/ eye irritation. Other ocular adverse reactions reported in less than 1% of patients include conjunctival hyperemia, eye discharge, eye dryness, eye pruritus and visual disturbance. The most frequently reported non-ocular adverse reactions included headache and rhinitis occurring in approximately 3% and 2% of patients respectively. Additional non-ocular adverse reactions reported in less than 1% of patients included asthma, nasal irritation and taste perversion. Some of these events were similar to the underlying disease being studied. The following adverse reactions have been identified during postmarketing use of RELESTAT™. <u><i>Eye disorders</i></u> Ocular hyperemia, lacrimation increased, eye pain, eye swelling, eyelid edema <u><i>Immune system disorders</i></u> Hypersensitivity reaction including symptoms or signs of eye allergy and skin allergic reaction DRUG INTERACTIONS No specific drug interaction studies have been performed with RELESTAT™ ophthalmic solution. No drug-drug interactions are anticipated in humans since systemic concentrations of epinastine are extremely low following ocular dosing. In addition, epinastine is mainly excreted as unchanged drug in humans which is indicative of minimal metabolism DOSAGE AND ADMINISTRATION The recommended dose for adults is one drop instilled in each affected eye twice daily, during the symptomatic period. There is no experience with the use of RELESTAT™ ophthalmic solution for more than 8 weeks. The contents of the bottle remain sterile until the original closure is broken by twisting the cap to pierce the dropper tip. To avoid contamination do not touch any surface with the dropper tip. If more than one topical ophthalmic medicinal product is being used, the medicinal products should be administered at least 10 minutes apart. <u><i>Elderly patients</i></u> RELESTAT™ ophthalmic solution has not been studied in elderly patients: Post-marketing safety data from the tablet formulation of epinastine hydrochloride (up to 20 mg once daily) indicates that there are no particular safety issues for elderly patients compared with adult patients. As such, no dosage adjustment is considered to be necessary. <u><i>Children and Adolescents</i></u> RELESTAT™ ophthalmic solution may be used in children and adolescents (3 years of age and older) at the same dosage as in adults.
INDICATIONS AND USAGE RELESTAT™ ophthalmic solution is indicated for prevention of itching associated with allergic conjunctivitis. CONTRAINDICATIONS RELESTAT™ ophthalmic solution is contraindicated in those patients who have shown hypersensitivity to epinastine, benzalkonium chloride, or any other ingredient in this product.		
WARNINGS RELESTAT™ is for topical ophthalmic use only and not for injection or oral use. PRECAUTIONS Information for Patients: Patients should be advised not to wear a contact lens if their eye is red. RELESTAT™ ophthalmic solution should not be used to treat contact lens related irritation. The preservative in RELESTAT™, benzalkonium chloride, may be absorbed by soft contact lenses. Patients wearing soft (hydrophilic) contact lenses should be instructed to remove contact lenses prior to administration of the solution and wait 10-15 minutes after instilling RELESTAT™ before reinserting soft contact lenses. Patients should be instructed to avoid allowing the tip of the dispensing container to contact the eye, surrounding structures, fingers, or any other surface in order to avoid contamination of the solution by common bacteria known to cause ocular infections. Serious damage to the eye and subsequent loss of vision may result from using contaminated solutions. Bottle should be kept tightly closed when not in use. Carcinogenesis, Mutagenesis, Impairment of Fertility: In 18-month or 2-year dietary carcinogenicity studies in mice or rats, respectively, epinastine was not carcinogenic at doses up to 40 mg/kg [approximately 30,000 times higher than the maximum recommended ocular human dose of 0.0014 mg/kg/day (MROHD) on a mg/kg basis, assuming 100% absorption in humans and animals]. Epinastine in newly synthesized batches was negative for mutagenicity in the Ames/Salmonella assay and <i>in vitro</i> chromosome aberration assay using human lymphocytes. Positive results were seen with early batches of epinastine in two <i>in vitro</i> chromosomal aberration studies conducted in 1980s with human peripheral lymphocytes and with V79 cells, respectively. Epinastine was negative in the <i>in vivo</i> clastogenicity studies, including the mouse micronucleus assay and chromosome aberration assay in Chinese hamsters. Epinastine was also negative in the cell transformation assay using Syrian hamster embryo cells, V79/HGPRT mammalian cell point mutation assay, and <i>in vivo/in vitro</i> unscheduled DNA synthesis assay using rat primary hepatocytes. Epinastine had no effect on fertility of male rats. Decreased fertility in female rats was observed at an oral dose up to approximately 90,000 times the MROHD. Pregnancy: Teratogenic Effects: Pregnancy Category C In an embryofetal developmental study in pregnant rats, maternal toxicity with no embryofetal effects was observed at an oral dose that was approximately 150,000 times the MROHD. Total resorptions and abortion were observed in an embryofetal study in pregnant rabbits at an oral dose that was approximately 55,000 times the MROHD. In both studies, no drug-induced teratogenic effects were noted. Epinastine reduced pup body weight gain following an oral dose to pregnant rats that was approximately 90,000 times the MROHD. There are, however, no adequate and well-controlled studies in pregnant women. The safety of epinastine hydrochloride tablets (10 or 20 mg once daily) has been evaluated in 11 pregnant women. No adverse drug reactions were observed and there was no influence of epinastine on the pregnant women, delivery, or newborn infants. Because animal reproduction studies are not always predictive of human response, RELESTAT™ ophthalmic solution should be used during pregnancy only if the potential benefit justifies the potential risk to the fetus.		

Colors:

	Black		Keyline
---	-------	---	---------

ARTWORK TEMPLATE
0106701

		File Upload Date: 26 Sep 2023 15:50 UTC		Version Number: 5		Artwork is created at 100%	
Brief ID: PROD-2023-001145		New Control Number: 20081064		PDS: N/A			
Size or Drawing Number: N/A				LAGN Drawing: 0106701			
Braille Code: N/A		Packaging Barcode Type: n/a		Packaging Barcode Number: n/a			
Variable Data Required: No							
Comments: Perforation required: No 2 pages Drop dieline, swatches, and art block before processing. Smallest Body text size: 7 pt Vendor allowed to add printer marks, as necessary.							